

Beth Israel Deaconess Medical Center
BIDMC Manual

Title: Policy On Interventions That Are Considered Futile or Potentially Inappropriate
Policy #: CP-26

Purpose: To ensure that medical interventions always respect the dignity and integrity of both the patient and any involved health professionals.

Policy Statement: The goal of medicine is to benefit the patient, and the process of delivering medical care should always respect the dignity and integrity of both the patient and the health care team providing care. It is the policy of the Beth Israel Deaconess Medical Center that no patient should be forced to undergo, nor should any physician or health professional be required to provide, an intervention that is **futile** or **inappropriate**.

A. Definitions

1. An intervention shall be considered **“futile”** if it has no chance of achieving the intended physiologic goal.
2. **“Patient’s goals”** shall mean “patient’s known goals, values or health care preferences (as expressed by the patient or their surrogate decision-maker).”
3. A medical intervention should be considered **“potentially inappropriate”** if the intervention has at least some chance of achieving the intended physiologic goal, but the physician or health care professional believes that (1) providing the intervention would be outside the standards or customs of the relevant medical specialty, and/or (2) providing the intervention would be outside the boundaries of accepted medical practice generally, and/or (3) competing ethical concerns justify not providing the intervention.
4. A medical intervention should be considered **“inappropriate”** if providing the intervention would be inconsistent with the standards or customs of the relevant medical specialty. If there are not published or otherwise known standards or customs applicable to the patient’s particular clinical circumstances (*e.g.*, no consensus regarding provision of the disputed intervention to someone clinically similar to the patient), then generally accepted standards of medical practice more broadly, as well as other competing ethical concerns, may be considered to determine if the medical intervention is inappropriate. The determination of appropriateness shall not be based on any of the following:
 - a. Bias or stereotypes about a patient's underlying or current disability;
 - b. Judgments that the individual will be a burden on others due to their underlying or current disability, including, but not limited to caregivers, family, or society; or
 - c. A belief that the life of a person with their underlying or current disability has lesser value than the life of a person without such a disability, or that life with

such a disability is not worth living.

B. General Application

In general, treatment decisions should be guided by the **patient's goals**, rather than the values or treatment preferences of the clinical team (*i.e.*, what the clinical team members would want for themselves or their loved ones in a situation similar to the patient's).

Accordingly, in undertaking the care of a patient, a BIDMC health care team should offer all available medical interventions that have a reasonable likelihood of meaningfully advancing the **patient's goals**, so long as such are appropriate under the circumstances. If, however, a point is reached where providing the medical intervention is inconsistent with the standards or customs of the relevant medical specialty, outside the boundaries of accepted medical practice generally, and/or there are competing ethical concerns which justify not providing the intervention, the intervention likely is **inappropriate** and the team will not be required to provide the disputed medical intervention. For example, if the patient will experience suffering that is disproportionate to the benefit the patient will experience by receiving the medical intervention, then the intervention likely is inappropriate. Similarly, if a requested intervention is determined to be **futile**, then the clinical team would not be required to, and should not, provide it. In either case, the physician generally will continue to care for the patient, ensuring that any suffering is minimized and the patient's dignity is respected.

C. Procedure When Attending Physician in Collaboration with the Interdisciplinary Team Believes Medical Intervention is Futile or Inappropriate

When an attending physician in collaboration with the interdisciplinary team has concerns that a medical intervention requested by the patient/surrogate may be futile or inappropriate, they should ask themselves the following questions to examine their position:

1. Do I believe this requested treatment would not further the patient's goals, as I understand them?
 - a. Have I confirmed my understanding of the patient's goals with patient/surrogate?
2. Do I believe this requested treatment is inconsistent with the standards or customs of the relevant medical specialty, outside the boundaries of accepted medical practice generally, and/or there are competing ethical concerns which justify not providing the intervention?
 - a. What is the basis for this opinion?
3. How comfortable would I feel if the rationale for my position were to be publicly reviewed?
4. What are the consequences to the patient, surrogate, team, or institution as a result of implementing this decision?
5. Am I sure that my belief has not been influenced by the patient's disability, age, race, ethnicity, religion, perceived quality of life, sexual orientation, gender identity, ability

to pay, socioeconomic status, perceived social worth, immigration status, incarceration status, housing status, or past or future use of resources, or other psychosocial factors?

The attending physician shall meet with the patient and/or the patient's surrogate decision-maker ("patient/surrogate") to explain why they believe the requested intervention should not be provided. Patients, surrogates, family members, and providers generally agree on appropriate pathways to achieve patient-centered goals. Even when not everyone involved completely agrees on the optimal plan of care, in most cases those involved are able to reach a plan of care that all can accept (*i.e.*, consensus). The clinical team should consider seeking involvement of individuals skilled in negotiation, communication, and/or conflict resolution (*e.g.*, Social Work, Palliative Care, Spiritual Care, Patient Relations, or the Ethics Support Service) in these discussions with the patient/surrogate to help reach agreement or resolve disagreement (*i.e.*, reach consensus). In rare cases, however, significant disagreements may persist despite best efforts to have open and collaborative communication among the care team and the patient/surrogate.

Support services, such as peer-to-peer support, debriefing sessions, and employee assistance programs, are available to clinicians to help address their moral distress resulting from the case and participating in the process outlined in this policy. Similarly, the patient/surrogate should be advised of available psychosocial supports they may need throughout the patient's course of care (*e.g.*, Patient Relations), including while the patient/surrogate is working through the process outlined in this policy.

D. Procedure When Disagreement Persists for an Intervention Determined to Be Futile

Physicians or other clinicians may receive a request from a patient/surrogate for an intervention that is **futile**, as that term is defined in this policy. In such cases, the clinicians should engage empathetically with the patient/surrogate to (i) understand why the patient/surrogate is requesting the intervention, (ii) correct any misunderstandings about the intervention, its intended physiologic goal, or its applicability to the patient, (iii) provide support, and (iv) explain why the requested intervention cannot achieve its intended physiologic goal for the patient and therefore will not be provided. Clinicians should consider seeking involvement of individuals skilled in negotiation, communication, and/or conflict resolution (*e.g.*, Social Work, Palliative Care, Spiritual Care, Patient Relations, or the Ethics Support Service) if disagreement persists. Clinicians also should ensure that Social Work is involved to help provide psychosocial support to the patient/surrogate. The attending physician may consider getting a second opinion to confirm their understanding that the requested intervention cannot achieve its intended physiologic goal.

E. Procedure When Disagreement Persists for an Intervention Considered Potentially Inappropriate

If all efforts to achieve consensus on an acceptable plan of care are not successful, the

following series of steps should be taken, each in a timeframe appropriate to the patient's clinical situation, with each step documented in the medical record:

1. Involve Individuals Skilled in Negotiation, Communication, and/or Conflict Resolution

To the extent not already involved, individuals skilled in negotiation, communication, and/or conflict resolution (*e.g.*, Social Work, Palliative Care, Spiritual Care, Patient Relations, or the Ethics Support Service) should be consulted to assist in efforts to reach agreement, or at least resolve the disagreement (*i.e.*, reach consensus).

If the disagreement persists despite their involvement (whether such individuals have been involved in the patient's case throughout the period of disagreement or initially got involved at this stage), then:

2. Notify the Patient/ Surrogate Regarding Process Outlined Herein

The patient's attending physician shall explain to the patient/surrogate the nature of the continued disagreement between the physician and the patient/surrogate. Physician also shall notify the patient/surrogate that the process outlined herein has been started and ensure that the patient/surrogate understands the process. Such communication shall include sharing with the patient/surrogate the written summary of the process outlined herein, attached as Exhibit A to this policy. A co-chair of the Ethics Advisory Committee and/or a member of the Ethics Support Service will be available to assist in explaining the process outlined in this policy to the patient/surrogate, if requested by the attending physician. Physician shall document in the medical record the content of this conversation and confirm that the written summary was provided to the patient/surrogate.

If the patient/surrogate wants to continue with the process, then:

3. Independent Second Opinion

The attending physician shall contact the Ethics Support Service ("ESS") to notify them of the continued disagreement, and the need to get an independent second opinion from an *ad hoc* committee of BIDMC attending physicians and other appropriate staff (*e.g.*, nursing, rehabilitation services) who are not currently engaged in the patient's care (but who are clinically active) and who represent leadership and expertise in or relevant to the clinical area of the disputed intervention (*e.g.*, nephrology for a disagreement about dialysis, critical care for a dispute about intubation, etc.) ("Specialist Committee"). ESS shall contact the appropriate unit/division leadership to notify them of the continued disagreement and the need for the unit/division leadership to identify the Specialist Committee members. Unit/division leadership shall consult (when appropriate) with the relevant Chief(s) of Service and/or the Chief Medical Officer when identifying members. The Specialist Committee shall review the patient's medical record and a physician member of the Specialist Committee shall examine the patient. The Specialist Committee also shall meet with the attending physician and/or involved social worker to understand the patient/surrogate's position and why they

continue to request the disputed intervention. Based on this investigation, its members' existing expertise and experience, their assessment of prognostic certainty in this case, review of relevant literature, and, as appropriate, conversations with colleagues at other leading healthcare institutions, the Specialist Committee shall determine whether providing the intervention would be outside the standards or customs of the relevant medical specialty. Documentation of the Specialist Committee's assessment shall be placed in the patient's medical record. The expectation is that the Specialist Committee will meet within twenty-four hours of the request, if possible.

- a. If the Specialist Committee agrees that providing the intervention would be *inconsistent* with the standards or customs of the relevant medical specialty, then the Specialist Committee shall meet with the attending physician to discuss the committee's conclusions and their reasoning. The attending physician shall inform the patient/surrogate of (1) the purpose of the Specialist Committee review and its composition, and (2) the Specialist Committee's conclusion. A member of the Specialist Committee will be available to assist in explaining the Committee's conclusions and reasoning to the patient/surrogate, if requested by the attending physician.
- b. If the Specialist Committee concludes that providing the intervention would be *consistent* with the standards or customs of the relevant medical specialty (*i.e.*, it does not agree with the attending physician's position), then the Specialist Committee shall meet with the attending physician to discuss the committee's conclusions and their reasoning. If, based on the committee's conclusion, the attending physician no longer objects to providing the intervention, then no change or further follow-up is necessary and the intervention shall be provided with the consent of the patient/surrogate. If, however, the attending physician continues to object to providing the intervention, then the relevant Chief of Service should be asked to try to identify an attending physician willing to serve in this role and provide the intervention. If another attending is willing to serve in this role and provide the intervention, the remainder of the process outlined herein is no longer necessary. If the Chief of Service cannot find another attending willing to serve in this role and provide the intervention, then this should be communicated to the patient/surrogate and the process outlined herein should continue.
- c. If the Specialist Committee concludes that there is no consensus regarding provision of the disputed intervention to someone clinically similar to the patient – for example, because there are not published or otherwise known standards or customs applicable to the patient's particular clinical circumstances, or there is inconsistency in how similarly situated patients are treated at BIDMC and/or other organizations – then the Specialist Committee shall meet with the attending physician to discuss the committee's conclusion and the basis for such. If, based on the committee's conclusion, the attending physician no longer objects to providing the intervention, then no change or further follow-up is necessary and the intervention shall be

provided with the consent of the patient/surrogate. If, however, the attending physician continues to object to providing the intervention, then the Review Committee shall be convened, pursuant to Step #5, below.

If disagreement persists, then:

4. Option to Transfer

The patient/surrogate should be offered the opportunity to seek transfer to a facility willing to provide that intervention. Appropriate BIDMC staff will provide reasonable assistance to the patient/surrogate to find such a facility. The patient/surrogate should be encouraged to seek additional support from a BIDMC Patient Relations Representative and/or the Ethics Support Service and/or Social Work in this and any subsequent steps described below.

If these efforts to transfer are not successful and the patient/surrogate continues to insist on the intervention, then:

5. Convening of a Review Committee

The Ethics Support Service person on-call, in consultation if possible with at least one Co-Chair of the Ethics Advisory Committee ("EAC") or designee, will initiate steps to convene, within a timeframe appropriate to the circumstances, a meeting of a review committee ("Review Committee"), generally consisting of the following individuals:

1. Chair or Co-Chair of the EAC or designee (Required)
2. Ethics Support Staff member (Required)
3. Chief(s) of the relevant medical or surgical service(s) or their designees not involved in the patient's care (Required)
4. Chief Medical Officer or designee (Required)
5. Chief Nursing Officer or designee (Required)
6. Director of Social Work or designee
7. Community member representative from the Patient Care Assessment Committee (PCAC)
8. Director of Spiritual Care or designee
9. Chief of Psychiatry or designee
10. General Counsel or designee

11. Representative from Diversity, Equity, and Inclusion Council, or such other hospital group that focuses on health care equity
12. Others deemed appropriate by the Review Committee or its chair (e.g., cultural liaison who can help navigate cultural issues that may be at play in the case).

The EAC representative will serve as moderator and neutral chair of the Review Committee. A quorum will consist of a minimum of six individuals: the five required committee members (see #1 - #5, above), including at least two physicians, and at least one community member or social support perspective (see #6, #7, #8, and #11, above). The quorum also must include at least one representative of the relevant medical or surgical service(s).

The Review Committee will consult with the following parties:

- a. The attending physician, who should explain the basis for deeming the intervention to be potentially inappropriate. The attending physician also should speak to the Specialist Committee's conclusions, which will be documented in the medical record. Other members of the clinical team should be asked to add details or to offer different perspectives and interpretations. The Review Committee and/or the attending physician may also request that a representative from the Specialist Committee speak to the Review Committee.
- b. The patient/surrogate (or family spokesperson(s)), who should explain the patient's perspective regarding the intervention under consideration, including their understanding of the patient's condition and prognosis, their hopes or expectations of benefit from the intervention, and their views about possible harm(s). The Review Committee should give the patient/surrogate sufficient time to explain their perspective. A BIDMC Patient Relations Representative and/or Social Worker should be available to the patient/surrogate before, during, and after this step.
 - i. If the patient/surrogate expresses, or it otherwise appears, that they are not comfortable meeting with the full Review Committee, even with the offered support of Patient Relations and/or Social Work, the Review Committee should offer to have an *ad hoc* subcommittee meet with the patient/surrogate instead, with such subcommittee reporting back to the full Review Committee.
 - ii. The patient/surrogate may decline to meet with the Review Committee or an offered subcommittee. Their decision not to meet with the Review Committee should neither be seen as undermining the integrity of the process set forth in this policy nor invalidating the Review Committee's recommendations, so long as good faith efforts have been made to include the patient/surrogate in the process. If the patient/surrogate declines to meet with the Review Committee, staff that have been supporting the

patient/surrogate (if any), such as Patient Relations or Social Work, should be invited to speak to the Review Committee about the patient/surrogate's perspective regarding the intervention under consideration.

After the discussions described above, the committee will meet alone to determine whether (i) the process described in this policy has been followed and performed appropriately, and (ii) the intervention is inappropriate. If the Specialist Committee concluded that providing the intervention would be inappropriate, pursuant to Section E3.a, above, the expectation is that the Review Committee similarly will find the intervention inappropriate, unless other competing ethical concerns justify providing the intervention. If the Specialist Committee concluded that providing the intervention would *not* be inappropriate, pursuant to Section E3.b, above, the expectation is that the Review Committee similarly will find the intervention appropriate, unless other competing ethical concerns justify *not* providing the intervention. If the Specialist Committee concluded that there is no consensus in the field regarding provision of the disputed intervention to someone clinically similar to the patient, pursuant to Section E3.c, above, the Review Committee shall consider whether providing the intervention would be outside the boundaries of accepted medical practice generally, and/or whether competing ethical concerns justify not providing the intervention. "Competing ethical concerns" might include, for example: inability to adequately control pain despite maximal efforts, evidence of the irreversible and imminent dying process, inequitable utilization of resources in times of resource constraint, maintaining the moral integrity of staff.

If the Review Committee does not conclude that the intervention is inappropriate, it may offer suggestions for identifying BIDMC staff that may be willing to assume responsibility for the patient's care. Ultimately, it is the responsibility of the Chief Medical Officer, in collaboration with the applicable Chief of Service, to find an attending physician now willing to provide the intervention in light of the Review Committee's conclusion. If the Review Committee does conclude that the intervention is inappropriate, it may offer suggestions to the involved clinical service(s) and BIDMC administration regarding steps that may be taken to ensure that BIDMC staff are not required to provide the intervention. Additionally, if the Option to Transfer was not offered to and discussed with the patient/surrogate, as described in Section E4, above (due to Specialist Committee's findings described in Section E3.c, above), it should be offered now.

One or more representatives of the Review Committee will communicate the committee's conclusion, any related suggestions, and its reasoning to the concerned parties, with documentation in the medical record. The Review Committee will designate the appropriate committee representatives, given the circumstances, to communicate with the concerned parties, and generally should include the EAC co-chair who chaired the Review Committee meeting. The attending physician and the committee representative will determine who will communicate the committee's conclusion to the patient/surrogate.

If the patient or surrogate are not in agreement with a determination by the Review Committee that the intervention is inappropriate, and they are unable or unwilling to arrange for transfer to another facility after a reasonable timeframe (not to exceed three business days from the

date of the Review Committee's decision, or such later date when the option of transfer to another facility is meaningfully discussed with the surrogate), they should be informed of their options for seeking, within a reasonable timeframe (not to exceed three business days), court involvement. Unless there are legal barriers to doing so, however, BIDMC administration shall take steps to support implementation of the committee's conclusion.

The Review Committee shall ensure that staff and patients/surrogates have been advised of the support services available to them, as described in Section C, above. In preparation for future applications of this policy, the Review Committee also should review how involved care team members were supported throughout the process outlined herein to ensure that the team members practiced, and the patient was cared for, in accordance with BIDMC's highest standards.

F. Treatment Plan During any Period of Disagreement

During any period in which there is disagreement about the appropriate plan of care, the patient's attending physician is ultimately responsible for determining what intervention(s) will and will not be offered. If there is uncertainty about this, the attending physician should consult, as appropriate, with the Ethics Support Service, the BILH Office of General Counsel, and/or the relevant Chief(s) of Service.

Reference:

Bosslet, G. T., Pope, T. M., Rubenfeld, G. D., Lo, B., Truog, R. D., Rushton, C. H., Curtis, J. R., Ford, D. W., Osborne, M., Misak, C., Au, D. H., Azoulay, E., Brady, B., Fahy, B. G., Hall, J. B., Kesecioglu, J., Kon, A. A., Lindell, K. O., & White, D. B. (2015). An Official ATS/AACN/ACCP/ESICM/SCCM Policy Statement: Responding to Requests for Potentially Inappropriate Treatments in Intensive Care Units. *American Journal of Respiratory and Critical Care Medicine*, 191(11), 1318–1330. <https://doi.org/10.1164/rccm.201505-0924st>

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Approved By:

Medical Executive Committee: 6/18/2025

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Eliminated:
References:

Exhibit A

Summary of Process for Patient/Surrogate
(attached)

Appendix 1
Meeting Guide
(attached)